

If you are thinking about surgery for pelvic organ prolapse, you and your surgeon will make several choices together. There is usually **no single “right” answer**. The best plan depends on your body, your health and what matters most to you. This sheet walks through the main decisions.

The Main Decisions

- **Repair or closure:** rebuild the support and keep the vagina open, or close the vagina (colpocleisis) if you no longer want vaginal intercourse.
- **Route:** through the vagina, or through the belly with small cuts (laparoscopic or robotic surgery).
- **Your uterus:** remove it (hysterectomy), or keep it and lift it into place (hysteropexy). Keeping it is often an option if it is healthy.
- **Support material:** your own tissue (native-tissue repair), or mesh placed through the belly (sacrocolpopexy). Mesh placed through the vagina for prolapse is no longer sold in the US.
- **Urine leakage:** whether to treat leakage at the same time, later, or not at all.

A Word on Hidden Leakage

A large prolapse can **hide** leakage with coughing or effort. The leakage may show up once the prolapse is fixed. This is called “occult” stress leakage. Your team may test for it by holding the prolapse up while your bladder is full. Adding a sling at the same time can lower the chance of leakage afterward. It also adds some risks, such as trouble emptying the bladder. Waiting and treating leakage later, only if it happens, is also reasonable.

Learn the Terms

Reconstructive

A repair that rebuilds support and keeps the vagina open.

Obliterative

A repair that closes the vagina (colpocleisis). Vaginal intercourse is no longer possible.

Hysteropexy

Lifting the uterus back into place without removing it.

Native-tissue repair

A repair using only your own tissue and stitches.

Sacrocolpopexy

A repair that places mesh through the belly to lift the vagina.

Occult stress leakage

Hidden leakage that may appear after prolapse is fixed.

Recurrence

When prolapse comes back after treatment.

How do I choose? Start with **your** priorities: how long the repair lasts, recovery time, keeping your uterus, your sex life, and how you feel about mesh. Your surgeon will explain what fits your body and health. The best choice is the one that matches your goals.

Questions to Ask Your Surgeon

- Which **procedure** do you suggest for me, and why?
- **Vaginal or belly** route: what is recovery like for each?
- Can I **keep my uterus**?
- Will you use **my own tissue or mesh**, and what are the risks?
- Should I have a **leakage procedure** now, later, or not at all?
- How likely is the prolapse to **come back**, and what then?

Also Worth Knowing

- A **pessary** is a non-surgical choice if you would rather avoid or delay surgery.
- Keeping stools soft, treating a long-term cough, and managing weight are good for your pelvic floor.
- After menopause, vaginal estrogen cream may ease symptoms after surgery. It has not been shown to make a repair last longer.

Getting Ready

- Bring a full medicine list. Tell your team if you take a **blood thinner**.
- Stop smoking. Arrange help at home.
- Most people can walk the first day and return to activity at their own pace. Sex waits until your team says so.

Call the office that is treating you if you have:

- A bulge that is **painful, bleeds, or has a sore** that does not heal
- Burning with urination, or a temperature of **100.4 °F (38 °C)** or higher

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. Prolapse surgery involves several choices. There is rarely one right answer.
2. The best plan fits your body, your health and what matters most to you.
3. A pessary is always an option if you want to avoid or delay surgery.